

PRIVATE HEALTHCARE INVESTMENT — RIYADH, KSA

# Should We Proceed With the \$200M Hospital Investment?

Final investment committee synthesis of the 150-bed premium orthopedic hospital opportunity — market, economics, structure, financing and forecast.

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Prepared for: Investment Committee

Week 12 — Project Structure & Delivery | Synthesis of Weeks 5–11

**150-bed**

Premium orthopedic hospital

**SAR 675mn**

Total project CapEx (Mid case)

**70/30**

Debt / equity, PPP structure

# Proceed — via a Partner-Structured, PPP-Financed Investment

*The economics work, the structure fits the client's capability, and the financing is bankable — conditional on fixing a proven cost-flex risk before scaling.*

## DECISION: PROCEED WITH CONDITIONS

Option B (Partner / JV) · Hybrid-revenue PPP · 70/30 debt-equity · SAR 202.5mn sponsor equity

1

### Economics clear the bar

39.1% stabilized EBITDA margin; DSCR of 1.91x vs. a 1.30x lender covenant; free cash flow positive in every year of every scenario.

2

### Structure matches capability

Partnering cuts investor CapEx ~60% (SAR 675mn → 270mn) and transfers construction, clinical and operating risk to an experienced operator.

3

### Growth plan is credible

Every driver in the 5-year forecast traces to a quantified Week 7 lever, stress-tested across 3 scenarios and 5 sensitivities.

## KEY RISK

Cost discipline is unproven on this asset. Three years of actuals show operating margin compressing 22.1% → 15.8% as OpEx grew ~1.5x faster than revenue — the opposite of the margin expansion the forecast relies on. Condition for proceeding: cost-flex controls validated before growth capital is scaled.

# What We Are Deciding Today

*Six weeks of workstreams converge on one investment-committee question.*

***“Should we proceed with this \$200M hospital investment?”***

## WHAT THE COMMITTEE DOES NOT NEED

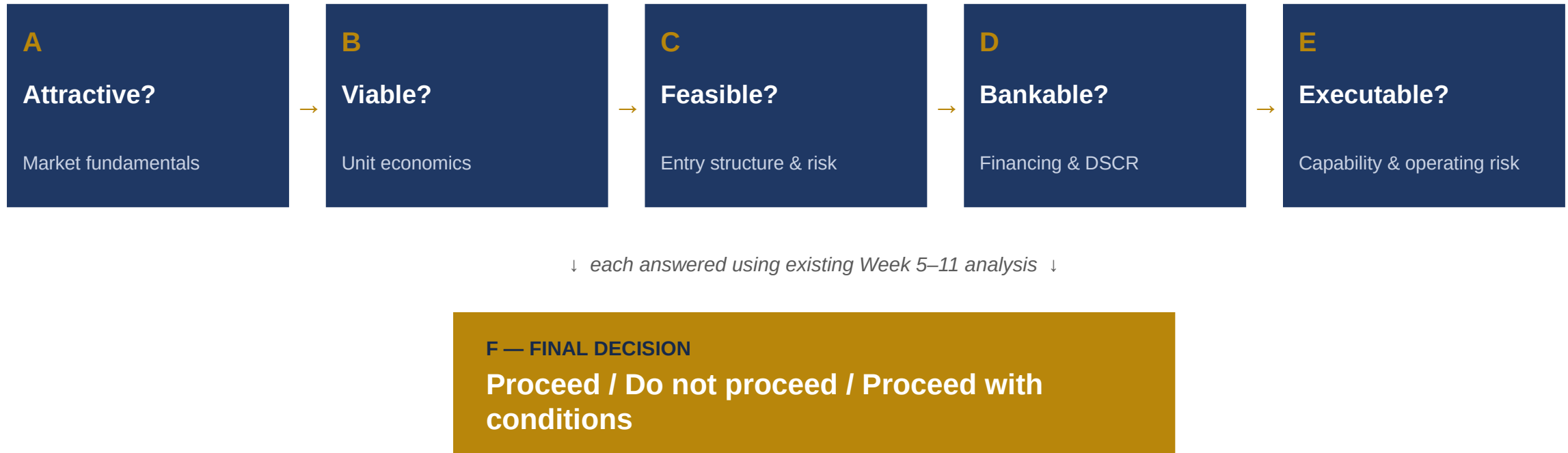
- × More analysis
- × More data
- × A menu of options

## WHAT THE COMMITTEE NEEDS

- ✓ A structured, prioritized answer
- ✓ A single, defensible recommendation
- ✓ Clear conditions for proceeding

# One Issue Tree, Six Questions, One Answer

*Everything from Weeks 5–11 is structured into five branches feeding a single decision.*

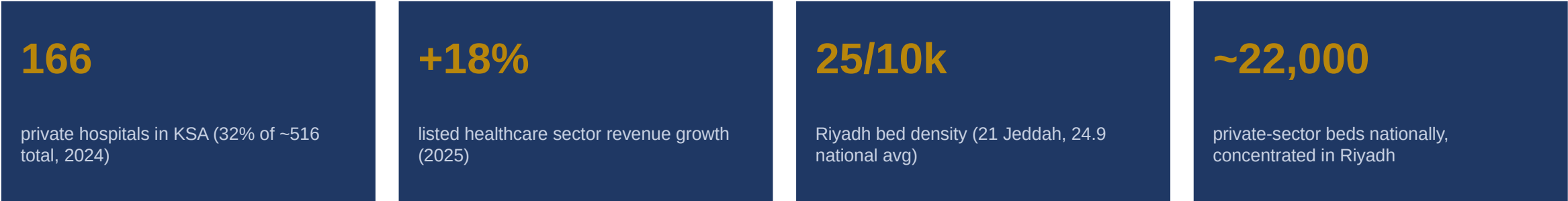


## SOURCE MAP

- Week 5 → unit economics (B)
- Week 6 → entry structure (C)
- Week 7 → growth strategy (feeds D)
- Week 8–9 → operating risk (E)
- Week 10 → financing & DSCR (D)
- Week 11 → forecast (D, F)

# A Private Healthcare Market in Structural Expansion

*Riyadh sits at the center of Saudi Arabia's private-hospital build-out — the right specialty, at the right time.*



## WHY ORTHOPEDICS, WHY NOW

Orthopedics is a high-value, implant-intensive, largely elective specialty — well suited to a premium private model with predictable scheduling, strong pricing power, and lower dependence on emergency/trauma infrastructure than a general acute hospital. Vision 2030's push to expand private-sector healthcare capacity is a direct tailwind.

*Market read: attractive — structural growth, favorable policy backdrop, and a specialty mix that plays to the asset's strengths.*

# Unit Economics Clear a Healthy Bar

Per-bed economics scaled to 150 beds — Mid case is the underwriting case.

| Metric                | Low   | Mid   | High  |
|-----------------------|-------|-------|-------|
| Total CapEx (SAR mn)  | 525   | 675   | 825   |
| Total CapEx (US\$mn)  | 140   | 180   | 220   |
| Revenue / yr (SAR mn) | 329   | 493   | 732   |
| EBITDA / yr (SAR mn)  | 89    | 193   | 357   |
| EBITDA Margin         | 27.0% | 39.1% | 48.8% |

## BENCHMARK POSITIONING (MID CASE)

|                       |                                                  |
|-----------------------|--------------------------------------------------|
| CapEx / bed           | Within range vs. Saudi general & int'l specialty |
| Revenue / bed         | Above market — 1.5–2.7x KSA sector average       |
| OpEx / bed            | Above market — implant-heavy COGS                |
| EBITDA margin (39.1%) | Within range vs. listed KSA peers                |

### KEY READ

CapEx is defensible against Saudi and international benchmarks. Revenue per bed is the swing factor — aggressive but justifiable given the implant-heavy, premium case-mix — and the single most important assumption to pressure-test in execution.

Economics verdict: viable — Mid case clears 39.1% EBITDA margin and 3.5-year payback at stabilized occupancy.

# Partner, Not Build Alone — the Structure Fits the Client

The client brings capital, not healthcare operating experience. Option B closes that gap directly.

| Metric                | Option A — Build & Operate       | Option B — Partner (Selected)          |
|-----------------------|----------------------------------|----------------------------------------|
| Investor CapEx        | SAR 675mn (\$180mn)              | <b>SAR 270mn (\$72mn)</b>              |
| Investor EBITDA       | SAR 193mn                        | <b>SAR 65mn</b>                        |
| EBITDA Yield on CapEx | 28.6%                            | <b>24.2%</b>                           |
| Simple Payback        | 3.5 yrs                          | <b>4.1 yrs</b>                         |
| Time to opening       | ~30 months                       | <b>~18 months</b>                      |
| Execution feasibility | Weak — no operating track record | <b>Strong — operator fills the gap</b> |

WHY OPTION B WINS

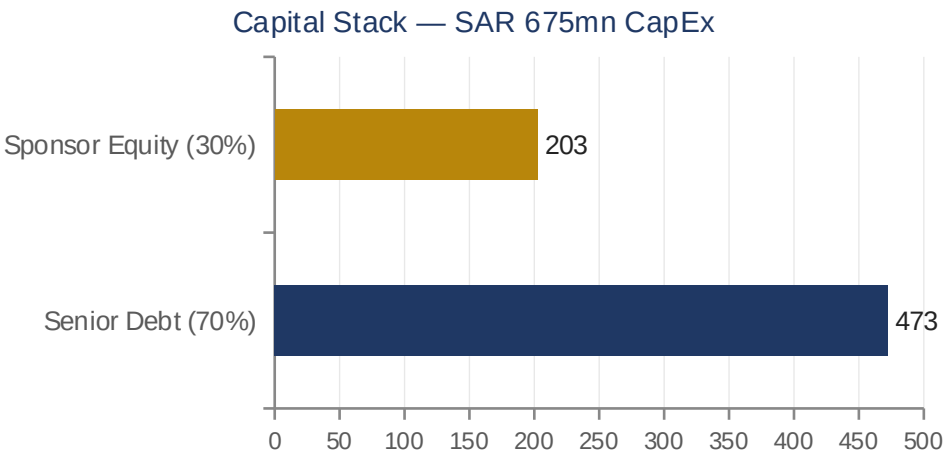
- ~60% less capital at risk (SAR 405mn freed for diversification)
- Transfers construction, staffing & clinical-quality risk to a proven operator
- ~12 months faster to first revenue
- Still a healthy 24.2% EBITDA yield on invested capital

STRUCTURE

Enter via a joint venture / management agreement with an experienced hospital operator: ~40% investor economic interest, operator runs clinical operations for a 6% management fee, governance rights follow ownership.

# The Deal Is Financeable: 70/30 PPP, DSCR Clears Covenant Every Year

A hybrid-revenue PPP structure gives lenders a predictable floor while keeping upside tied to real demand.



### DSCR VS. 1.30X LENDER COVENANT

| Year | 1     | 3     | 5     | 10    |   |
|------|-------|-------|-------|-------|---|
| DSCR | 1.91x | 3.23x | 3.65x | 5.09x | 7 |

Minimum DSCR of 1.91x in Year 1 (weakest year) — a 0.61x cushion above the 1.30x covenant, widening every year as debt amortizes and revenue escalates.

### REVENUE STRUCTURE — HYBRID

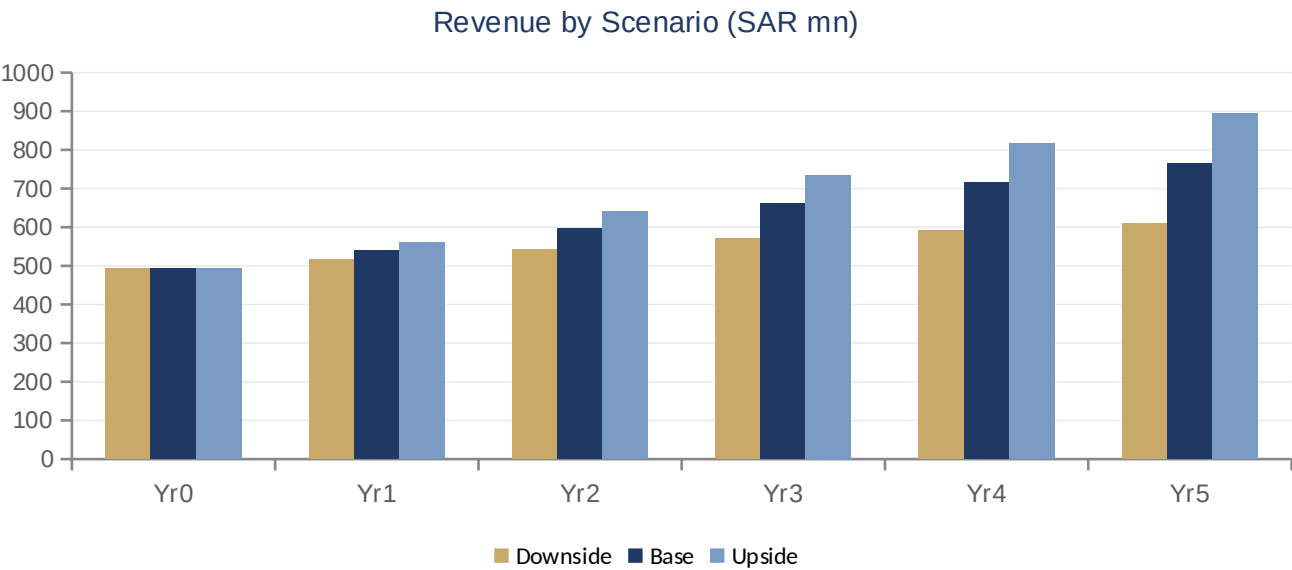
30% government availability payment (CPI-indexed, contracted public-patient capacity) + 70% patient/insurer billings on private volume — de-risks the base for lenders while preserving upside.

### BANKABILITY VERDICT

Yes. Coverage, predictability and risk allocation all clear the bar — construction and operating risk sit with parties equipped to manage them; government retains only regulation and licensing.

# A Credible Path to +55% Revenue Growth by Year 5

Four quantified levers — not a single bet — carry the growth plan from Year 0 through Year 5.



FOUR GROWTH LEVERS — YEAR-3 CONTRIBUTION (BASE CASE, SAR mn)

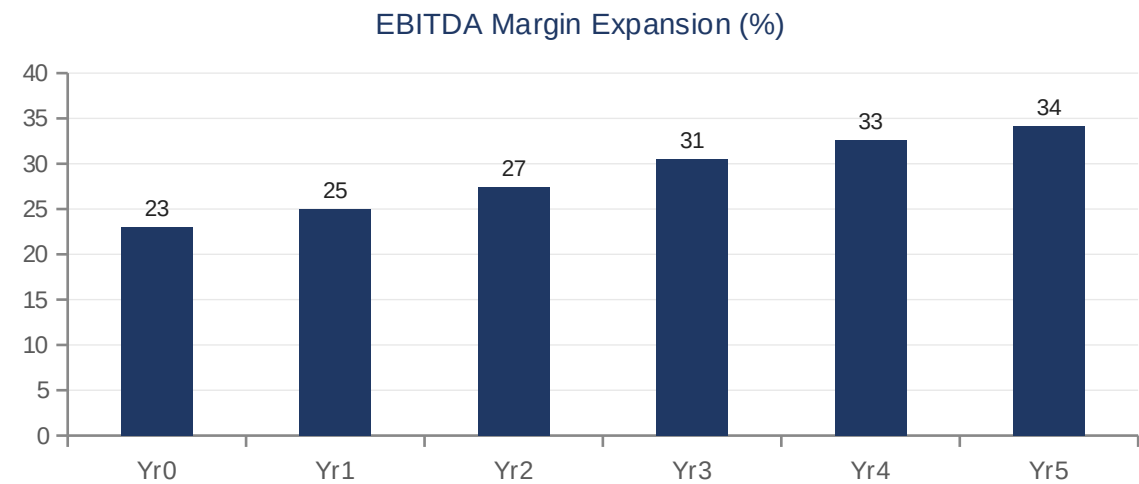
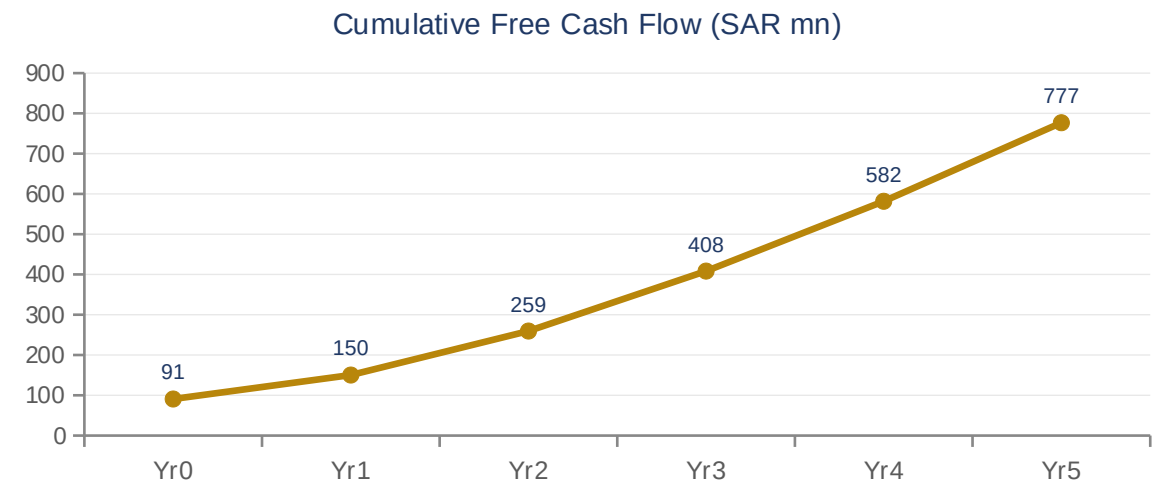
|                             |      |
|-----------------------------|------|
| Pricing Optimization        | 48.3 |
| PRIORITIZE                  |      |
| New Service Line            | 38.0 |
| LONGER HORIZON              |      |
| Occupancy & Throughput      | 33.8 |
| PRIORITIZE                  |      |
| Outpatient/Rehab Throughput | 22.8 |
| PARALLEL                    |      |

SAR 636mn Base Case Year-3 revenue (+29.0%) — essentially on the +30% target; extended to SAR 765mn (+55.2%) by Year 5.

Priority: Levers 1 (Occupancy) & 3 (Pricing) underwrite the Base Case alone — Lever 4 pursued in parallel, Lever 2 (New Service Line) is the multi-year upside driver.

# Self-Funding Through the Full Forecast Horizon

Base Case: revenue to net income, margin expansion, and free cash flow — Year 0 to Year 5.



| Metric              | Year 0 | Year 5 | Change  |
|---------------------|--------|--------|---------|
| Revenue (SAR mn)    | 493.0  | 765.0  | +55.2%  |
| EBITDA (SAR mn)     | 113.4  | 260.9  | +130.1% |
| Net Income (SAR mn) | 90.7   | 198.6  | +118.9% |
| EBITDA Margin       | 23.0%  | 34.1%  | +11.1pp |

CREDIBILITY CHECK

Every driver traces to a Week 7 lever. 3 scenarios, 5 sensitivities. No single flex breaks Year-5 EBITDA. FCF positive in every year of every case.

# What Actually Matters — Five Insights, Not Fifty Numbers

*Synthesizing six weeks of analysis into the drivers that determine the decision.*

|   |                                                      |                                                                                                                                      |
|---|------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------|
| 1 | <b>Unit economics are attractive but revenue-led</b> | Revenue per bed (1.5–2.7x sector average) — not CapEx — is the assumption carrying the case. CapEx is defensible against benchmarks. |
| 2 | <b>Partnering is the right entry point</b>           | The client's capital strength and lack of healthcare operating experience make Option B (JV/partner) the fit — not full ownership.   |
| 3 | <b>The deal is bankable as structured</b>            | A 70/30 hybrid-revenue PPP clears the 1.30x DSCR covenant with room to spare in every year, from Year 1 onward.                      |
| 4 | <b>Growth is diversified, not a single bet</b>       | Four levers — occupancy, pricing, new service line, throughput — combine to reach the target; no lever alone carries the plan.       |
| 5 | <b>Cost discipline is the swing factor</b>           | The forecast's margin expansion assumes fixed-cost leverage. Historical evidence on this exact cost base points the other way.       |

# What Could Change the Answer

*Top risks, ranked by how directly each threatens the investment case.*

|                               |                                                                                                                                                                                                                            |                                                                                                                   |
|-------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------|
| <b>Cost flex (critical)</b>   | 3-yr actuals on this asset show OpEx growing ~1.5x revenue's rate; margin compressed 22.1% → 15.8%. A comparable Riyadh group shows the same failure pattern (-16.5% margin) when cost structure isn't matched to pricing. | <i>Validate &amp; fix cost-flex controls before scaling growth spend; gate new spend on margin stabilization.</i> |
| <b>Pricing achievability</b>  | Revenue/bed assumption sits 1.5–2.7x the KSA sector average — the single biggest driver of the range.                                                                                                                      | <i>Full feasibility pricing survey across Riyadh private competitors before financial close.</i>                  |
| <b>Occupancy ramp-up</b>      | Payback and DSCR assume stabilized occupancy; comparable hospitals take 2–4 years to get there from a ~30% Year-1 start.                                                                                                   | <i>Model Year 1–3 explicitly; align debt covenant headroom to the ramp curve.</i>                                 |
| <b>Specialist recruitment</b> | New Service Line depends on hiring 2–3 specialist surgeons by Year 2 — a hiring/credentialing risk, not a demand risk.                                                                                                     | <i>Start search now; bridge with locum/visiting-surgeon model.</i>                                                |
| <b>Operator dependency</b>    | Partner structure transfers execution risk but creates dependency on the operator's standards and incentives.                                                                                                              | <i>Negotiate quality-control rights and brand-standard covenants in the JV/management agreement.</i>              |

# Proceed — Partner-Structured, PPP-Financed

Conditional on validating pricing and fixing cost-flex discipline before growth capital is scaled.

*“We recommend proceeding with the Riyadh 150-bed orthopedic hospital investment, entered via a joint-venture/management-agreement partner and financed as a hybrid-revenue PPP at 70/30 debt-equity — because the economics are bankable, the structure matches the client's capability, and the growth plan is credible — conditional on validating premium pricing and proving cost-flex discipline before scaling.”*

**SAR 202.5mn**

Sponsor equity required

**1.91x**

Minimum DSCR (Yr1) vs. 1.30x covenant

**34.1%**

Year-5 EBITDA margin, Base Case

**SAR 776.7mn**

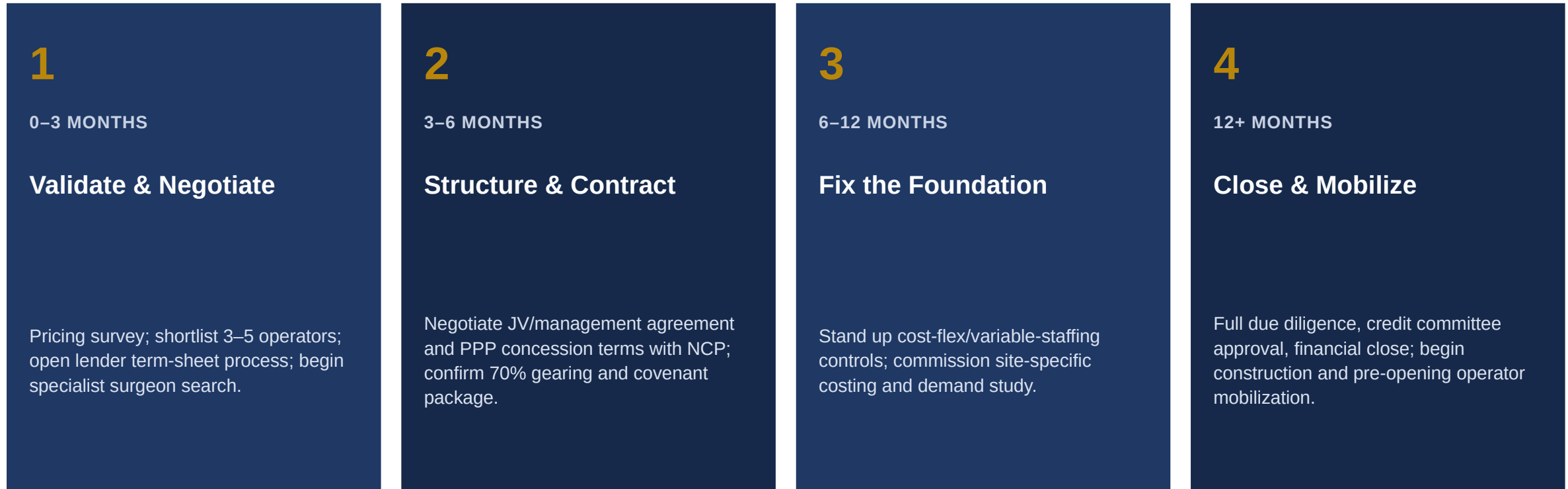
Cumulative FCF by Year 5, Base Case

## CONDITIONS FOR PROCEEDING

1. Validate premium pricing via a Riyadh feasibility survey. 2. Implement and prove cost-flex controls before scaling growth spend. 3. Secure operator shortlist & specialist recruitment pipeline before financial close.

# From Decision to Financial Close

*A sequenced path — validation and negotiation run in parallel, not in series.*



*Owner: sponsor deal team, in coordination with the National Center for Privatization & PPP and the selected operating partner.*

# Every Number Traces to a Prior Workstream

Full line-level detail, formulas and source notes are in the companion IC Decision Summary workbook.

| Week | Workstream                | What It Contributed to This Decision                                             |
|------|---------------------------|----------------------------------------------------------------------------------|
| 5    | Unit Economics Screen     | Per-bed CapEx/Revenue/OpEx, benchmark comparison, EBITDA margin & payback ranges |
| 6    | Build vs. Partner         | Option A/B comparison; recommendation to enter via JV/management agreement       |
| 7    | Revenue Growth Strategy   | Four quantified growth levers; +30% 3-year revenue target and scenario sizing    |
| 8–9  | Performance Diagnosis     | Cost-flex risk evidence from 3-yr actuals; comparable-market precedent           |
| 10   | PPP & Bankability         | 70/30 capital structure, hybrid revenue model, DSCR & risk allocation            |
| 11   | 5-Year Financial Forecast | Full P&L, CapEx, cash flow; 3 scenarios, 5 sensitivities, credibility assessment |

Companion file: *Week12\_IC\_Decision\_Summary.xlsx* — Tabs: *IC Decision Summary, Unit Economics, Build vs Partner, Revenue Growth, Performance Diagnosis, PPP & Bankability, 5-Yr Forecast.*